

Lecture: Billing and Coding for OMT

Sheldon C Yao, DO, FAAO
Professor, Chair
Department of OMM

Do.
Make.
Heal.
Innovate.
Reinvent the Future.

Session Objectives

1. **Define** essential terminology crucial for comprehending proper documentation and billing in medical practice.
2. **Compare and contrast** the distinctions among ICD-10 codes, Evaluation and Management (E/M) codes, and Current Procedural Terminology (CPT) codes
3. **Determine** the requisite documentation elements for an office visit, encompassing osteopathic manipulative treatment (OMT)
4. **Explain** the process of billing for an office visit and OMT as a procedural service, incorporating the appropriate use of Modifier -25
5. **Provide** an overview of all components necessary for billing OMT services, including somatic dysfunction documentation by regions treated, techniques utilized, and patient response to treatment

Framework – Importance medical billing and coding

- The backbone of the healthcare revenue cycle, ensuring payers and patients reimburse providers for services delivered.
- Medical billing and coding translates a patient encounter into the languages healthcare facilities use for claims submission and reimbursement.
- Medical coding involves extracting billable information from the medical record and clinical documentation.
- Utilizing these codes, medical billing generates insurance claims and bills for patients, streamlining the reimbursement process.
- Understanding the fundamentals of medical billing and coding ensures physicians to maximize allowable reimbursement for delivering quality care.
- Errors in coding and billing can have severe repercussions, impacting revenue streams, diminishing value, and potentially jeopardizing medical licenses.



Starts with patient doctor encounter and documentation

- Accurate and complete clinical documentation during the patient encounter is critical for medical billing and coding.
- The golden rule of healthcare billing and coding departments is, “Do not code it or bill for it if it’s not documented in the medical record.”

**NEW YORK INSTITUTE
OF TECHNOLOGY**

College of Osteopathic
Medicine



Key terminology

NEW YORK INSTITUTE
OF TECHNOLOGY

College of Osteopathic
Medicine

- International Classification of Diseases, 10th revision (ICD 10)
 - A code set providers use to report **medical diagnoses including Osteopathic diagnoses** on all types of claims for services rendered in the United States.
 - These are codes used in the **Assessment** section of the note
- Current Procedural Terminology code (CPT)
 - A medical set of descriptive terms and identifying codes used to report medical, surgical, and diagnostic procedures
 - These are codes that we use to bill for a service we provide in the clinic, hospital, operating room, etc.
- Evaluation and Management code (E/M)
 - A category of CPT codes used for the purpose of billing
 - Used to bill your history and associated evaluation and management services for every patient.

Documentation Guidelines

NEW YORK INSTITUTE
OF TECHNOLOGY

College of Osteopathic

- Each patient encounter should include:

- S - Subjective

Chief complaint & History

- O – Objective

Vitals, PE, OSE, Labs/imaging

- A – Assessments

Summary of your diagnosis (ICD10)

- P – Plan

Treatment - additional work up, labs, studies, OMT, referrals, exercises, education lifestyle modifications

01/01/201X - Pod 1 AM	
Room #: 01	
Enc. #: 01	
Routing #: 1	
NBOME ID #: 111111	
SOAP Note	
S	
George Payne is a 45-year-old Caucasian right handed male who presents to the Family Medicine office with a complaint of right-sided chest discomfort for one week. The pain began after moving into a new house. He denies any injury to his chest. The pain is sharp like a knife and constant, and he rates it 3-4/10. The pain radiates through to the back intermittently. It is relieved with a hot shower, ibuprofen, or lying on his left side, but made worse with lifting boxes and taking deep breaths. He has had heartburn in the past, but says that this pain is different and never had anything like this previously. He is concerned that this could be related to his heart. ROS: No palpitations, shortness of breath, diaphoresis, nausea, or vomiting Pmhx: heartburn occasionally Surghx: tonsillectomy Meds: Ibuprofen three times a day ALL: NKDA Fam Hx: Mother living and in good health, Father died of heart attack at age 60 Sochx: Married with 3 children. No tobacco, 6-pack of beer on weekends occasionally, marijuana in high school. Occupation is a roofer.	
O	
Vitals: 70 inches 190 lbs 27.3 kg/m2 BMI 132/80 BP 98.4 Temp 80 HR 16 RR Gen: mildly anxious male in mild distress, occasionally touching R chest wall Heart: regular, no murmurs, S3 or S4 Lung: clear bilateral anterior and posterior, slight increase in pain with deep breath T spine: Paravertebral tenderness R T4-8 tenderness Chest wall: Tenderness with palpation anterior chest wall mid-clavicular line at rib 4	
A	P
1. Thoracic somatic dysfunction 2. Costochondritis 3. Rib fracture-unlikely 4. Anginal equivalent-doubt 5. Family history of heart disease	1. OMT: Balanced ligamentous tension technique. Schedule for additional OMT if pain does not resolve 2. NSAIDS/ moist heat 3. X-ray-rib films r/o fracture 4. EKG r/o acute coronary syndrome vs. prior event

The Note: Subjective/History

**NEW YORK INSTITUTE
OF TECHNOLOGY**

College of Osteopathic
Medicine

- When evaluating a patient for a complaint where OMT is performed:
 - Pertinent HPI must be present
 - Example: For a patient with headache - we need onset, progression, provocation/palliation, what have they tried, associated symptoms aura, sinus pressure/pain and location if present. (OPQRSTA)
 - Past medical history
 - Includes medications, supplements, and allergies
 - Past surgical history
 - Family history
 - Depending on the chief complaint, you need to ask about: Parents, siblings, and children histories
 - Social history
 - Occupation, living situation, ETOH, Drugs, Exercise, etc.

The Note: Objective/Physical Exam

**NEW YORK INSTITUTE
OF TECHNOLOGY**

College of Osteopathic
Medicine

- In addition to vital signs and key physical exam findings when performing and billing for OMM you **MUST** document your OMM somatic dysfunction findings and the treatment applied.
- Ideally it should be clearly delineated - listed by regions.

Sample OSE documentation

NEW YORK INSTITUTE
OF TECHNOLOGY

College of Osteopathic
Medicine

OSE (Osteopathic Structural Examination)

- Head – OAFSRRL (3) – tx with MET (1)
- Cervical – C2FSRR (2) – tx with HVLA – (0), right SCM spasm (1) tx with MFR (0), C1 right posterior TP (2) tx with CS (0)
- Rib – right 1st rib inhalation dysfunction (2) – tx with Still's technique (0)
- Thoracic – T2-5NSRRL (1) – tx with FPR (0)
- Key mild (1), moderate (2), severe (3)

** (Or can list findings in OSE and list OMT utilized in the Plan as per NBOME sample)

The Note: Assessment/Diagnoses

**NEW YORK INSTITUTE
OF TECHNOLOGY**

College of Osteopathic
Medicine

- All diagnoses:
 - Also known as International Classification of Disease (ICD) codes
 - Currently on the 10th revision
 - All have a letter and number designation
 - Example: G43.119 Migraine with aura, intractable, without status migrainosus
- For every visit where OMT is performed:
 1. List all your Medical Diagnoses that you obtained a history and performed a physical exam on first (G43.119)
 2. After you have listed your medical diagnoses, then you will list the ICD 10 code(s) that corresponds to the region(s) of the body where manipulation was performed.
 - Remember: There are 10 body regions- Head, Cervical, Thoracic, Rib, Upper extremity, abdomen, lumbar, sacrum, pelvis, lower extremity
 - Example: M99.00 Segmental and Somatic Dysfunction of the Head region

Assessments – ICD 10 codes

NEW YORK INSTITUTE
OF TECHNOLOGY

College of Osteopathic
Medicine

List your assessment with most pertinent patient presentation.

Somatic dysfunction (SD) should never be only code of the first diagnostic code. A SD does not justify an office visit.

Sample:

Assessment: (ICD-10):

1. G43.119 Migraine with aura, intractable, without status migrainosus
2. M54.2 Cervicalgia (Neck Pain)
3. W56.12 Struck by sea lion
4. M99.00 Somatic dysfunction of head region
5. M99.01 Somatic dysfunction of cervical region
6. M99.02 Somatic dysfunction of thoracic region
7. M99.08 Segmental and somatic dysfunction of rib cage



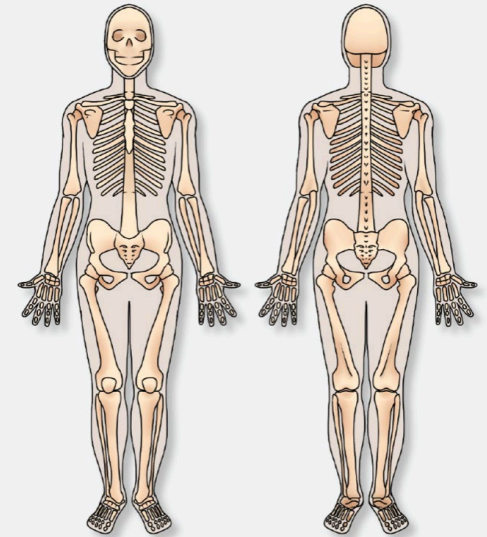
OMM ICD10 Codes

ICD-10-CM codes for reporting Somatic Dysfunction are located in chapter 13: Diseases of the Musculoskeletal System and Connective Tissue (M00 – M99).

- M99.00 Segmental and somatic dysfunction of head region
- M99.01 Segmental and somatic dysfunction of cervical region
- M99.02 Segmental and somatic dysfunction of thoracic region
- M99.03 Segmental and somatic dysfunction of lumbar region
- M99.04 Segmental and somatic dysfunction of sacral region
- M99.05 Segmental and somatic dysfunction of pelvic region
- M99.06 Segmental and somatic dysfunction of lower extremity
- M99.07 Segmental and somatic dysfunction of upper extremity
- M99.08 Segmental and somatic dysfunction of rib cage
- M99.09 Segmental and somatic dysfunction of abdomen and other regions

**NEW YORK INSTITUTE
OF TECHNOLOGY**

College of Osteopathic
Medicine



Breakdown of OMT Regions

Head

- Cranial Somatic Dysfunction
- Facial Somatic Dysfunction
- OA

Cervical

- AA
- C2-7

Thoracic

- T1-12
- **Thoracic inlet

Ribs

- **Thoracic Inlet
- Rib 1-12
- **Sternoclavicular joint

Upper Extremity

- **Sternoclavicular joint
- Acromioclavicular joint
- Glenohumeral joint
- Ulna
- Radial head
- Carpals

Lumbar

- L1-5
- Quadratus Lumborum

Abdomen

- Celiac
- Mesentery/Visceral

Sacrum

Pelvis

- Pubic bones
- Innominate

NEW YORK INSTITUTE OF TECHNOLOGY

College of Osteopathic
Medicine

Lower extremity

- Femur/hip
- Knee
- Tibiotalar joint
- Talocalcaneal joint
- Metatarsals

**List Joints or areas that can be documented in multiple regions. Pick the region that makes the most sense to you and be consistent. **

The Note: Plan

- If an evaluation was performed and during your medical decision making process you feel that OMT would help your patient and you perform this procedure on your patient:
 - You need to document it as such in your plan so that an auditor or colleague will know that both were performed.
 - **EXAMPLE: “Based on today’s physical exam, OMT was performed to the above regions using MFR, BLT, HVLA, Still Technique, counterstrain...”**
 - The “above areas” are the somatic dysfunction regions listed in the assessment section of the note that also correspond to the OSE exam findings in each of the regions listed in the objective section of the note.

Procedure Note

- OMT is a procedure – Must include a post procedure note
- Sample -
- Patient evaluated and treated with OMT as noted, tolerated procedure well, noted decreased pain, increased ROM, etc... post procedure.
- Ideal to note any functional gains post procedure

Getting paid for the office visit and a procedure

- The office visit itself is the evaluation and management code (E&M)
- The E&M is justified by your documentation of your findings (Hx & PE) and assessments (ICD10 codes)
- Any additional procedures (ie OMT, injections, biopsy) as long as separate and distinct services are performed
- The procedures are documented as **CPT codes** which are justified by associated assessments (ICD10 codes)

Basic Components of E/M Coding Based on 2021 Guidelines:

- Time

- Counseling
- Coordination of care dominates service

- Medical Decision Making

- Number & complexity of problems addressed
- Amount &/or complexity of data analyzed and reviewed
- Risk of complication, morbidity &/or mortality

**NEW YORK INSTITUTE
OF TECHNOLOGY**

College of Osteopathic
Medicine

CPT® CODE	2021 OFFICE & OTHER OUTPATIENT SERVICES TIME New	CPT® CODE	2021 OFFICE & OTHER OUTPATIENT SERVICES TIME Established
99201	N/A	99211	N/A
99202	15-29	99212	10-19
99203	30-44	99213	20-29
99204	45-59	99214	30-39
99205	60-74	99215	40-54

MDM

E/M Code	Level of MDM (2 out of 3 Elements)	Number and Complexity of Problem Addressed	Amount and/or Complexity of Data to be Reviewed and Analyzed	Risk of Complications and/or Morbidity or Mortality of Patient Management
99202: 15-29 minutes 99212: 10-19 minutes	Straightforward	1 self-limited or minor problem	Minimal or None	Minimal risk of morbidity from additional diagnostic testing or treatment
99203: 30-44 minutes 99213: 29-29 minutes	Low	2 or more self-limited or minor problem Or 1 stable chronic condition Or Acute, uncomplicated illness or injury	Must meet requirements of I of the 2 categories Category 1: Test and Documents Any combination of 2 from the list below. Or Category 2: Assessment requiring an independent historian(s)	Low risk of morbidity from additional diagnostic testing or treatment
99204: 45-59 minutes 99214: 30-29 minutes	Moderate	1 or more chronic illnesses with exacerbation, progressions, or side effects of treatment Or 2 or more stable chronic illnesses Or 1 undiagnosed new problem with uncertain prognosis Or 1 acute illness with systemic symptoms Or 1 acute complicated injury	Must meet requirements of I of the 3 categories Category 1: Test and Documents, or Independent Historian(s) Any combination of 3 from the list below. Or Category 2: Independent Interpretation of Test Independent interpretation of a test performed by another physician or other health care professional Or Category 3: Discussion of Management or Test Interpretation Discussion of management or test interpretation with external physician or other health care professional	Moderate risk of morbidity from additional diagnostic testing or treatment Examples - Prescription drug management - Decision regarding minor surgery with identified patient or procedure risk factors - Decision regarding elective major surgery without identified patient or procedure risk factors - Diagnosis or treatment significantly limited by social determinants of health
99205: 60-74 minutes 99215: 40-54 minutes	High	1 or more chronic illnesses with severe exacerbation, progression, or side effects of treatment Or 1 acute chronic illness or injury that poses a threat to life or bodily function	Must meet requirements of 2 of the 3 categories Category 1: Test and Documents, or Independent Historian(s) Any combination of 3 from the list below. Or Category 2: Independent Interpretation of Test Independent interpretation of a test performed by another physician or other health care professional Or Category 3: Discussion of Management or Test Interpretation Discussion of management or test interpretation with external physician or other health care professional	High risk of morbidity from additional diagnostic testing or treatment Examples - Drug therapy requiring intensive monitoring for toxicity - Decision regarding elective major surgery without identified patient or procedure risk factors - Decision regarding emergency major - Decision regarding hospitalization - Decision not to resuscitate or to de-escalate care because of poor prognosis



OMT Billing

OMT is a manual treatment focused on addressing somatic dysfunctions of neuromuscular structures and it's associated with medical conditions and its effects on other body systems.

- **OMT is a PROCEDURE.**
- This occurs in addition to completing an evaluation of the patient including obtaining a pertinent history, completing a physical exam, and determining whether OMT is appropriate as a part of the medical decision-making process.

OMT Billing – CPT codes for procedures

- Billed based on the number or body regions treated
 - **10 Regions:** Head (includes the OA & hyoid), cervical, thoracic, rib, lumbar, abdomen, sacrum, pelvis (innominates & pubic bones), upper extremities, and lower extremities
 - Remember: even though most patients will have two upper extremities-they do not count as separate regions.
 - CPT Codes:
 - 98925: OMT to **1-2 body regions**
 - 98926: OMT to **3-4 body regions**
 - 98927: OMT to **5-6 body regions**
 - 98928: OMT to **7-8 body regions**
 - 98929: OMT to **9-10 body regions**

25 Modifier

- Definition: a billing code that is used when separately identifiable services are provided on the same day of service.
- Therefore, in an office visit encounter where OMT is performed, you would bill for an E/M code , a 25 modifier, and a separate CPT code for the OMT performed.

Billing Information:

Visit Code:

99213 Office Visit, Est Pt., Level 3. Modifiers: 25

Procedure Codes:

98927 OSTEOPATHIC MANIPULATION.

- Exception to the rule:
 - If you evaluate the patient and determine the need for OMT on one day and have the patient return on a different day specifically for OMT.
 - Plan: “Based on previous PE, OMT was performed to the above regions using...”

Putting it together

- Office visit. Patient seen and evaluated, OMT performed for headache
- E/M: 99213 (est patient, mod complex)
- Diagnosis (ICD-10):
 1. HA (R.51) [SD should never be only of 1st dx code – does not justify office visit]
 2. M99.00 Somatic dysfunction of head region
 3. M99.01 Somatic dysfunction of cervical region
 4. M99.02 Somatic dysfunction of thoracic region
 5. M99.08 Segmental and somatic dysfunction of rib cage
- Procedure (CPT): 98926 - OMT 4 regions
- Add -25 modifier → 99213 -25 mod 98926



Key pointers

- When reporting Evaluation and Management (E/M) services along with osteopathic manipulative treatment (OMT) performed on the same day, remember to add the -25 Modifier to the E/M code.
- Comprehensive documentation of the patient's history, including details such as the chief complaint, onset, frequency, duration, and other relevant factors, is essential for accurate coding and billing.
- Document the presence of somatic dysfunction observed during the examination.
- Provide a detailed account of the specific regions treated with OMT, ensuring clarity regarding the anatomical areas addressed.
- Describe all techniques employed during the OMT session and include information on how the patient responded to the treatment. This may involve noting the patient's tolerance, any adverse reactions, or improvements observed post-treatment.

Summary

- Proper understanding of coding and billing practices for medical visits is crucial for securing accurate reimbursement.
- Inadequate documentation and incorrect billing can have serious consequences.
- Governmental guidelines governing coding and billing are subject to frequent changes, necessitating continuous updates.
- Physicians bear the ultimate responsibility for ensuring adherence to coding and billing standards.
- Thorough documentation of OMT, precise coding for SD assessments, and meticulous billing procedures are essential for securing proper reimbursement.

References

- AOA-Osteopathic Billing and Coding. (n.d.). Retrieved January 23, 2020, from <https://osteopathic.org/practicing-medicine/business-of-medicine/osteopathic-billing-coding/>
- CPT evaluation and management office or other outpatient & prolonged services code & guideline changes. (2021) Retrieved from: <https://www.ama-assn.org/system/files/2019-06/cpt-office-prolonged-svs-code-changes.pdf>
- Snider, KT, & Jorgensen, DJ. Billing and Coding for Osteopathic Manipulative Treatment. *The Journal of the American Osteopathic Association*. (2009); 109(8), 409. <https://www.degruyter.com/document/doi/10.7556/jaoa.2009.109.8.409/html>

Lecture and Lab Feedback Form:

**NEW YORK INSTITUTE
OF TECHNOLOGY**

College of Osteopathic
Medicine

<https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448FWYXREL4R>